

Title

On-site hepatitis C care reduces injection drug use and related risk in people who inject drugs: findings from an accessible care model trial

Abstract

Structural barriers significantly limit hepatitis C treatment access among people who inject drugs, yet evidence on innovative care-delivery models remains limited. A randomized 1:1 assignment trial enrolled 165 HCV RNA-positive individuals with recent injection drug use and randomized participants to either on-site hepatitis C care (n=82) delivered directly at a harm-reduction program, or standard referral pathways (n=83). The on-site care model provided clinical assessment, treatment initiation, psychosocial support, and medication management without requiring external facility referral. Participants attended structured follow-up interviews at 3, 6, 9, and 12 months, with standardized assessments of injection patterns, high-risk injection practices, and medication-assisted treatment engagement. By 12-month follow-up, on-site care participants reported statistically significantly greater reductions in injection drug use frequency compared with standard-referral participants. High-risk injection behaviors also declined more substantially in the accessible-care arm. Rates of opioid-use-disorder treatment engagement appeared similar between study arms. These findings demonstrate that removing structural and administrative barriers to hepatitis C care can meaningfully reduce injection-related behaviors and may amplify public-health impact in affected populations.